6.17 Acute Pancreatitis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "Severe pain in the upper abdomen going to my back." • Rates pain 9/10 **Objective** • Raised lipase • Guarding • Vomiting	**ACTUAL** Acute pain related to pancreatic inflammation as evidenced by severe epigastric pain radiating to the back.	• Pain at or below 3/10 within 24 hours.	• Give regular opioid analgesia as prescribed. • Let him sit leaning forward, or lie on his side with knees bent. • Nil by mouth at first, as ordered; start feeding when ordered. • Pass an NG tube if vomiting persists, as ordered.	• Achieved when pain is controlled.
Subjective • "I am very thirsty." **Objective** • HR 118/min, BP 100/60 mmHg • Urine 20 mL/hour	**ACTUAL** Inadequate fluid volume related to third-space fluid shift and vomiting as evidenced by tachycardia, hypotension and low urine output.	• Urine at least 0.5 mL/kg/hour within 12 hours.	• Give IV fluids as prescribed. • Measure urine output hourly. • Follow calcium, watching for tetany, and potassium. • Take vital signs hourly.	• Outcome reached with adequate output.
Subjective *No specific complaint* **Objective** • Pancreatic inflammation • Glucose 190 mg/dL	**RISK** Risk for physical injury as evidenced by risk factors: hyperglycemia and hypoglycemia associated with acute pancreatic injury.	• Blood glucose stays on target.	• Check blood glucose every 4–6 hours. • Give insulin as ordered.	• Considered met while glucose is on target.

* **References for this care plan:** [32], [35], [81], [125]. Full entries are listed in the References section of the CPMN book.